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PSYCHOSOCIAL ASSESSMENT REPORT

Confidential – Prepared at the Client's Request for Submission to U.S. Citizenship and Immigration Services

Client	Marcia Evadne Whitfield
Date of Birth	03/14/1984 (age 41)
Client Address	418 Prospect Street, Apt. 3B, East Orange, NJ 07017
USCIS A-Number	[A-NUMBER]
Referral Source	Primary care physician
Dates of Contact	Intake 06/12/2025; follow-up sessions 06/26, 07/10, 07/24, 08/07, 08/21, 09/04, and 09/18/2025 (8 sessions total, 50–90 minutes each)
Evaluator	Gregory T. Hale, LCSW
Date of Report	09/26/2025

1. REASON FOR REFERRAL AND PURPOSE OF REPORT

Ms. Whitfield was referred to this writer in June 2025 by her primary care physician after she reported persistent low mood, insomnia, weight loss, and anxiety following the breakdown of her marriage. She has remained in individual outpatient treatment since that time. At Ms. Whitfield's request, this report summarizes my assessment, her reported history, my clinical findings, and my diagnostic impressions, for use in connection with her self-petition under the Violence Against Women Act (VAWA).

2. SOURCES OF INFORMATION

- Clinical interviews with Ms. Whitfield at intake and over seven follow-up sessions (approximately 9 hours of direct contact).
- Standardized screening instruments: Patient Health Questionnaire-9 (PHQ-9), Generalized Anxiety Disorder-7 (GAD-7), and PTSD Checklist for DSM-5 (PCL-5), administered at intake and again on 09/18/2025.
- Mental status examinations conducted at each session.

This writer did not interview Mr. Lattimore and did not observe the events described below. The history of the relationship is based on Ms. Whitfield's report. Her account was consistent in its essential details across sessions, and her affect was congruent with the content she described.

3. BACKGROUND AND PSYCHOSOCIAL HISTORY

Ms. Whitfield is a 41-year-old woman born and raised in Spanish Town, St. Catherine, Jamaica. She describes a stable childhood in a close extended family and reports no history of abuse in her family of origin. She completed secondary school in Jamaica and later trained as a caregiver. She came to the United States in August 2018 with her first husband, who held a research position. That marriage ended in an uncontested divorce in 2021, which she describes as sad but not abusive. She has no children.

Ms. Whitfield works full-time as a home health aide. She reports no prior mental health treatment, no psychiatric hospitalizations, and no history of alcohol or drug use. Before the events described below, she was socially active, attended church regularly, and maintained close relationships with her cousin in

Brooklyn and with a longtime friend in East Orange.

4. RELATIONSHIP HISTORY AS REPORTED BY THE CLIENT

Ms. Whitfield met Vernon Lattimore in October 2021 through a mutual friend. She describes a warm courtship, and the couple married on July 15, 2022, in East Orange, New Jersey, after which she lived with him at his apartment on Prospect Street. She describes the first year and a half of the marriage as largely happy. Shortly after the wedding she learned that Mr. Lattimore had concealed that he could barely read. She reports feeling hurt by the secrecy but supportive of him.

Ms. Whitfield reports that the relationship changed in early 2024. Mr. Lattimore lost his job in February 2024 and concealed this from her for several weeks. When she confronted him, he shouted at her and pointed his finger in her face at close range, which she experienced as frightening. Over the following months, she reports, he:

- stopped working while expecting her to pay all household expenses and to perform all cooking, cleaning, and laundry;
- repeatedly called her “stupid,” “useless,” and “worthless,” disparaged her cooking, and told her that no other man would want her;
- stayed out overnight and publicly humiliated her in front of his friends when she went looking for him in June 2024;
- pressured her into sexual relations when she declined, by becoming angry and accusatory, at a time she believed he was being unfaithful;
- spat in her face during an argument over money in August 2024;
- withdrew household funds for his own purposes and concealed debts that resulted in the loss of the couple’s 2024 tax refund; and
- told her that she depended on him for her immigration status and that he could have her “sent back,” then failed to appear for two scheduled immigration interviews in April 2025.

Mr. Lattimore moved out of the home without notice on May 17, 2025, removing property and funds, and has not responded to her attempts at contact. Ms. Whitfield reports that she did not contact the police during the marriage because of shame, a belief that verbal mistreatment would not be taken seriously, and fear that involving the police would worsen the situation at home and harm her immigration case. This pattern of non-disclosure is common among individuals subjected to coercive control and emotional abuse.

5. CURRENT SYMPTOMS

Ms. Whitfield reports persistent sadness and tearfulness most days since mid-2024; loss of interest in church and social activities; initial and middle insomnia (typically 4–5 hours of sleep per night); reduced appetite with an approximate 15-pound weight loss over 2024; fatigue; difficulty concentrating at work; and pervasive feelings of shame and worthlessness, often expressed in the same words Mr. Lattimore used toward her.

She also reports intrusive memories of the August 2024 spitting incident and of his shouting; an exaggerated startle response to raised voices; physiological anxiety and hypervigilance when she hears keys at the apartment door, even after his departure; avoidance of reminders of the relationship, including the street where his friend lives; and emotional numbing. She denies current or past suicidal ideation, intent, or plan,

and denies homicidal ideation.

6. STANDARDIZED MEASURES

Instrument	Intake (06/12/2025)	Re-test (09/18/2025)	Interpretation
PHQ-9 (depression)	19	14	Moderately severe at intake; moderate at re-test
GAD-7 (anxiety)	15	11	Severe at intake; moderate at re-test
PCL-5 (posttraumatic stress)	49	41	Above the clinical threshold (31–33) at both administrations

7. MENTAL STATUS EXAMINATION (MOST RECENT, 09/18/2025)

Ms. Whitfield presented as a neatly groomed woman who appeared her stated age. She was cooperative and engaged, with good eye contact that diminished when discussing the abuse. Speech was soft but normal in rate and volume. Mood was described as “tired and down”; affect was constricted and tearful when recounting specific incidents, and congruent with content. Thought process was linear and goal-directed. There was no evidence of psychosis. She denied suicidal and homicidal ideation. She was alert and fully oriented; attention and memory were grossly intact. Insight and judgment were good.

8. DIAGNOSTIC IMPRESSIONS (DSM-5-TR)

F32.1	Major Depressive Disorder, single episode, moderate
F43.10	Posttraumatic Stress Disorder
Z69.11	Encounter for mental health services for victim of spouse or partner psychological abuse

9. CLINICAL OPINION

Based on my assessment over eight sessions, Ms. Whitfield’s presentation is consistent with that of an individual who has been subjected to sustained emotional and psychological abuse by an intimate partner. Her symptoms began during the marriage, and specifically after the onset of the mistreatment she describes in early 2024. She has no prior psychiatric history and no other identified stressor that would account for them. Her account was detailed, internally consistent, and accompanied by appropriate emotional responses, and I found no indication of exaggeration or symptom fabrication.

In my professional opinion, the pattern Ms. Whitfield describes, including degradation, economic control, sexual coercion, threats related to her immigration status, and the spitting incident, constitutes a pattern of psychological abuse and coercive control. This abuse is the primary cause of her current depressive and posttraumatic symptoms.

10. TREATMENT AND RECOMMENDATIONS

- Continue weekly-to-biweekly individual psychotherapy using trauma-focused cognitive behavioral therapy (TF-CBT).
- Referral to a psychiatric provider for medication evaluation, which Ms. Whitfield is considering.

- Safety planning was completed in June 2025 and reviewed in September 2025. Ms. Whitfield is encouraged to maintain contact with her support network, particularly her cousin and longtime friend.

Ms. Whitfield has shown modest improvement since beginning treatment, as reflected in her re-test scores. Her prognosis is fair to good with continued treatment, a stable living situation, and the security of a resolved immigration status.

I certify that the information contained in this report is true and accurate to the best of my professional knowledge. I am available to provide additional information upon request with the client's written authorization.

09/26/2025

Gregory T. Hale, LCSW Date

Licensed Clinical Social Worker
New Jersey License No. [LICENSE NO.]